

MONICA NURSING STUDY CENTER

Maternity

Study Guide

Organized review material

Prepared for study and printing

Maternity

Nagele'r Rule:

Day + 7 Board: Marzo 4
Month + 9

GTPAL

- # of gestation
- Preterm 20-36 week
- Term 37-42 week
- abortion before 20 week
- live (Empezar siempre por este)

After 20 weeks

- After 20 weeks 1 cm x week until 37 weeks
- +/- 2 = ok
- Call Dr if +2 o -2
- Fundal height more than expected for gestational age could be:
 - Hydratiform
 - Twins

● Nutritional Requirements

Add 300 K cal- Pregnant

Add 500 K cal- breastfeeding

- Diet:

- High Calcio (Milkor yogurt)
- High Iron (Shellfish Vit C to fix)

Avoid:

- Sword fish
- Shark
- Keig Mackerel
- Tuna
- Mahi Mahi
 - Acid Folic replacement
 - * 400 mcg x day
 - * 600 mcg x day

● Weight during pregnant

- Normal BMI= 18.5-25 = 25-30 pounds during pregnant
- Under weight BMI: 18.5 = ↑ until 40 pounds
- Overweight BMI> 25= ↑ until 25 pounds

Until 20 week

+ 3-5 pounds

After 20 week

+ 1 pound x week

TORCH

T= Toxoplasmosis – Avoid cat feces, gardening and ingestion of unpasteurized milk or undercooked meals.

O= Others: HIV

- No internal fetal monitoring if Mom is HIV positive
- * Breastfeeding is contraindicated
- * Vaginal delivery if viral load less than 1000

Early Diagnostic

- Polymiraze chain reaction test

Nota: Breastfeeding contraindicated

- HIV
- Galactosemia
- Leukemia or Lymphoma type 1 virus

Diagnostic Studies**1. Leukocyte Esterase or nitrates positive (bacterial infection)**

Nota: lymphocytes = viral infections

2. Ultrasound:

- abdominal: Full bladder
- Transvaginal: Empty bladder

3. Biophysical profile

Noninvasive (20 weeks)

To check for Fetal well being

3. Chorionic Villus Sampling (invasive)

Nota: After Any Invasive procedure if mother = Rh negative administer RHOGAM (doesn't matter dad)

4. Alpha Fetoprotein

- 15-17 weeks

5. Amniocentesis (Invasive)

- ❖ L/E= 2:1= OK=Lung Maturity
- ❖ With empty bladder
 - L/E= less than 2:1= No lung maturity

6. Nitrazine Test

- Nitrazine strip or swab turn BLUE= positive for amniotic fluid

7. Fibronectin test

- 16-20 weeks= normal expect fibronectin in vaginal secretion

8. CBC Count, HB, Hematocrit

- Expect CBC 15 000-16 000
- During Labor: 20 000 - 25 000

9. Blood type

- if mother RH positive = is ok
- if Mother RH negative and father RH negative= is ok
- if mother RH negative and father RH positive= Do:
 - Indirect Coombs test to the mother
 - Before 28 week
 - If negative = Administer Rogan to the mom at 28 weeks
 - Do indirect coombs test after birth to the mom
 - If negative= Administer Rhogam to the mother, Within 72h of birth
 - If positive = OK

Discomfort during pregnant*Intervention*

1. Eat dry crackles, protein bar and stay on bed at least 15 mint after (dry carbohydrate).
2. Small and frequently feeding
3. Antiemetic:
 - Dimenhydrinate, Dramamine
 - If hyperemesis gravidarum=
 - Metabolic acidosis
2. Backache
 - Use low heel shoes
3. Constipation
 - Treatment = Water, Fiber, Walking
4. Leg Cramps = Due to low Ca
 - Increase Calcium intake (1 L/ daily)
 - Feet dorsiflexion
 - Do not massage
5. Pytalism (escupidera)
 - Suck hard candies
 - Use lip balm

Bleeding during pregnant

1. Hydatidiform Mole "No FHR"
 - Gestational Trophoblastic disease (Crece exagerado)
 - Grape like Cluster (Racimo de uva se suelta como forma de tela por la vagina)

Assesment:

1. No fetal tone
2. Fundal height greater than expect
3. ↑↑↑ HCG = chorionic gonadotropin = toomuch elevated
4. Sign of pre-eclampsia
5. Snowstorm patter on ultrasound - Tormenta de nieve

Intervention

- a. (D& C) (legrado diagnostic)
- b. HCG monitored every 1-2 weeks until normal pregnancy level
 - After that every 1-2 months for 1 year
 - If persistent elevated more than 6 months - Hysterectomy = Chemotherapy

2. Ectopic pregnancy*Cause*

1. Inflammatory pelvic Disease

Assessment

1. Amenorrhea (4-8 weeks)
2. Unilateral lower quadrant abdominal pain

Rupture = life threatening

1. Hypovolemic shock
 - hypotension + tachycardia

Intervention

1. IV fluid
2. Prepare for surgical intervention
3. Administer methotrexate if is stable patient

3. Abortion

<i>Type</i>	<i>Assessment</i>	<i>Nursing Consideration</i>
1. Threatened (Amenaza)	-Cervix close	-HCG studies
2. Inevitable or imminent (after pain= se abrio el cuello)	- Cervical dilation	
3. Incomplete	- Exposure of part of product of conception - More risk for infeccion "Quedo Resto"	
4. Complete	- Cervix close or near to close	- Methylergonovine or hemabate
5. Missed Abortion (Stillborn) Murio el feto dentro de la barriga	- Cervix is closed - High risk for infection	

Nota: Como se que una amenaza de aborto es un aborto inminente = cuando las HCG estan decreased

<i>Placenta Previa</i>	<i>Abrupto Placentario</i>	<i>Placenta Acreta</i>
- Painless, bright red - Uterus Soft	- Dark red vaginal bleeding - Very painful - More common during fetus delivery	- Occur when the placenta (After birth) grow into uterine wall- normally after child birth the placenta separate from the uterine wall. In case of placenta acreta it remains attached too deeply into the uterus.

Gestational Hypertension (after 20 week)

- Detected first time during pregnant = $>140/90$

<i>Pre- Eclampsia mild</i>	<i>Warning sign of eclampsia</i>	<i>Eclampsia Seizure</i>
-BP $> 140/90$ -Proteinuria = 2+ - Puffy fingers	1. Epigastric pain (Priority) - RED FLAG 2. Severe headache 3. Blurred vision 4. Clonus 5. HELLP Syndrome	- Rolling eye

H= Hemoptysis

E= elevated liver enzymes

L

L

P

= Low platelets

Emergency Priority (A correr)

Intervention Pre-eclampsia - Eclampsia

- RR: 16-20 = Ok
- Magnesium Sulfate ($MgSO_4$) - therapeutic level = 4-7 in OB

Magnesium Sulfate intoxication or toxicity = >7

(Para tratar pre-eclampsia, eclampsia)

1. Respiratory depression
 - Stop $MgSO_4$ + administer calcium gluconate
2. DTR= Normal =2

<i>Nonstress test (ultrasound)</i>	<i>Contraction stress test or oxytocin challenge</i>
- Empty Bladder	- La IV es la mas exacta

- 2/15/15/20 = Reactive = 20 = good finding = Acceleration of FHR= ok	- If positive (Mal) - If Negative (Bien)
--	---

Acceleration of FHR

- Reassuring pattern
 - Todas suben
 - Todas quedan por encima del 120

Early Deceleration

- Mirror image: cuando sube el pico de contraccion de la mama, baja el FHR pero no mas de 100
- At the begining of the contraction or at same time

Late Deceleration (Priority = No Good) (Key= After)

- Non-reassuring
- Fetal heart rate decelerated AFTER peak of uterine contraction
- Remain over 100 FHR

Intervention:

S top Oxytocin
T urn position to left or side lateral
O xigeno
P hysica or call Dr

Variable Deceleration

- Is cause by umbilical cord compression/ Por debajo de 100/ Perdida de liquido amniotico aprieta el cordon

Cause :

- Prolapse through vagina

Priority:

- When FHR repeatedly decrease less than 70 b/min and persist at this level for at least 60 seconds = Sufrimiento Fetal = Non reassuring

Intervention:

1. Change position:
 - Knee chest position
 - Recumbert

Umbilical Cord Prolapsed

- Do not reintroduce the umbilical cord (if mention towel roll= Ok)
 If prolapsed through vagina:
 - Cover or wrap moist sterile dressing

Labor**True Labor**

1. Contraction increased with ambulation
2. Pain lower back radiated to the abdomen "gridling"
3. Progressive cervical dilation

Leopold's maneuvers**Abdominal palpation** = Use to determine Fetal presentation, lie position and engagement

1. (Fundus Grip) ver que hay en el fondo
 - Button's (Brech) = Soft, immovable, large (Nalgas up)
 - Vertedor occipitals = Hard, small, movable (cabeza up)
2. (Espalda -manos/pie)
 - Small parts: (knots or irregulars)
 - Fetal back: (Smooth and hard)
3. (to determine presentation part/ fetal position)
4. (engagement)

Stations:**Ex: Tiene 10 cm de dilatacion, 90% effacement and -2 station****8 cm dilation, 90 % effacement and +2 station = esta es la priority****Fettal Back:**O- en el medio viene de **CABEZA**S- en el medio viene **SENTADO****Ex: ROA = Right lower quadrant****Stage of Labor****Stage 1**

- Start with the true labor
- End with the full dilation = 10 cm and 100% effacement
- ❖ **Latent Phase**
 - 0-3 cm of dilation

Board: husband watch football game in this stage

Nota: una vez que se rompe la membrana ya no puede caminar y debe estar en lithotomy position

- ❖ **Active phase**
 - Cervical dilation 4-7 cm

Intervention:

1. Effleurage (cosquilla)
2. Offer fluids and ice chips

- ❖ **Transition phase**
 - 45-90 second in duration

Complication

- Respiratory Alkalosis (Se va el CO₂)
 - Paresthesia (Lip and tip of fingers)
 - Mouth stiffness = (boca apretada)
 - Dizziness

Intervention

- Respiratory techniques to prevent Respiratory alkalosis = Respiratory Pattern
 - Pan-Pan-Blow
 - Hee-Hee-Whow

Board: Cuando se pone a paciente en lithotomy?= at the end of 1st or beginning 2do = when urge to push

Stage Second

Signs:

1. Urge to push or bearing down
2. Increased in bloody show
3. Bulging perineum and Anus

SATA Board

Stage Third

- Start with baby born until placenta expulsion

Signs

1. Gush of blood

Stage Fourth

1. *Vital sign*

- increased temperature = expect = 100.3 - 100.4 = Due to mild dehydration
- Expective bradycardia

2. *Check fundus*

- At the umbilicus or below, at the midline and firm
- Boggy 1-2 cm above the umbilicus, and deviated to the right = Full bladder
- Boggy 2 cm above the umbilicus
 - Massage

How to massage:

- One hand remains cupped against uterus at the level of SP to support uterus

3. **Check for Hemorrhage**

- More 500 ml/ 24 h for vaginal delivery
- More 700/24 h for C-Section

Rubra

- Fresh odor (0-3 days)
- Follow up if more than 3 days

If 5 days and have rubra= Call Dr

Secrosa

- Brownish pink with fleshy odor (4-10 days)

Alba

- Yellow to white 11-14 days
- If pad saturate in less than 1 hour = Hemorrhage = Call Dr
- Check the last time que se lo cambiaron.

4. **Dry and suction infant**

5. **Place blanked**

Post-partum (After 4h from birth)

- Gently cleans the vulva with surgigator or peri bottle

Post- partum Discomfort

1. After birth pain = Expect

Stimulate by:

- a. Breast feeding
- b. Oxytocin

2. Perineal Discomfort

- Ice pack during first 24H
- After 24H = sitz bath for 20 min 3-4 times/day, Water warm

Nota: In case of laceration= Avoid Enema and Suppositories

Breast

- Football position or belly to belly
 - Cheeks should be full and rounded
 - The best indicator of proper milk transfer is audible swallowing “K-ah” sound=Board
 - *Plugged Duct*
 - Heat and massage
 - *Nipple Irritation*
 - Clean with water (No Soap) and dry
 - Apply breast milk to the nipple and areola after each feeding and air dry
 - *Expression of mil*
 - Out of refrigerator - 4H
 - Refrigerated = 2-3 days
 - Freezer = 2 weeks
 - Deep Freezer = 2-6 months
- (Plastic pomito donde lo almacenas: Board)

Psychosocial Adjustment

- Touching baby face with fingertips (Board)

Phases of Adjustment

<i>Taking in</i>	<i>Taking Hold</i>	<i>Letting Go</i>
- 1-2 days after birth - Dependency - Only preoccupied with self-own needs (food and sleep) - Hesitate to make decision - Refuse to take any decision	- 3-10 days after birth (acepta la sugerencia de la suegra) - Open to instruction	- 5-6 weeks after birth - Assume a new roll responsibility

1. Post - partum complication

<i>Post partum Blue 3-7 days postpartum</i>	<i>Post-partum depression More than 2 weeks</i>	<i>Post-partum Psychosis At any time</i>
Intervention - Emotional Support	1. Anhedonia = loss of joy Treat: 1. SSRI- except for: Fluoxetine= Contraindicated in Breast Feeding Paroxetine = Choice	- Emergency = hospital admission - Personal history of psychotic disease

2. Hematomas*Cause:*

- Trauma-Forceps
- Bulbar hematoma (Board)
 - Pain in the vulva (Key word - Vaginal pain)

Assessment

- Abnormal severe vulvar or vaginal pain

3. Endometritis

- Rubra persistent > 3 days with foul smelling

Intervention (Hospital-admission)**4. Mastitis***Treatment*

- Continue lactando en el lado de la mastitis porque sino empeora

5. Shoulder distocia

- Turtle Sign

Intervention

- Mc Robert Maneuver (Flexionar las rodillas la pecho)

Complication:

- Clavicle Fracture da Erb's Palsy
 - * Paralysis or limpness of the shoulder, arm and elbow (finger movement non affected)
 - * Numbness, tingling (burners and stingers)

6. Rupture of the uterus (life treatenning)

- *Cause*

- C-Section (History)

- *Assessment*

- * Contraction sudden stops (Bajan las contracciones de 90-26, es decir muy rapido)

7. Uterine Inversion No. 1 Priority

- *Assessment:*
 - Depression in the fundal area (se forma un hueco)

8. Premature rupture of membrane

- Internal fetal monitoring = Ok
- Contraindicated
 - Vaginal Examination (Si se pone oxytocin porque ya rompio membrana)

Diabetes Gestational (+ 140)

- Screening 24-28 weeks
- Gold standart test
 - 1 hours glucose challenge test

Intervention

1. Diet and exercise (Main treatment)
2. Regular insulin (RRR) (IV) (Subcutanea)